

answer-key

Answer Key — Not For Upload

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For each case ID, this lists the expected classification (Present / Absent / Unclear) per policy requirement, and why. Requirement numbers refer to the numbered list in each policy document under `policies/`.

MRI Lumbar Spine (`policies/01-mri-lumbar-spine-policy.md`)

PA-2026-71442

All requirements should classify as **Present**. - Req 1 (conservative therapy): PT summary documents 8 weeks, dated, with modalities. - Req 2 (necessity statement): clinical note explicitly states rationale. - Req 3 (neuro exam): motor/sensory/reflex/SLR findings are specific and complete. - Req 4 (prior imaging): radiology report present, within 12 months. - Req 5 (red-flag screen): explicitly and affirmatively addresses all red flags.

PA-2026-71889

Expected: **Absent** on Req 4 and Req 5. Present on Req 1, 2, 3. - Req 4: no X-ray/CT report of any kind is included anywhere in this case's documents. - Req 5: the clinical note gives a reason for ordering imaging but never addresses cauda equina, infection, fracture, or malignancy — nothing to affirmatively screen against. - No conservative-therapy bypass exception is invoked or applicable.

PA-2026-72015

Expected: **Absent** on Req 1, 3, 4, 5. - Req 1: the only "physical therapy" mentioned is for an unrelated right shoulder injury, completed before this back pain even started — not lumbar-directed conservative care. - Req 3: straight-leg-raise was deferred, not performed — no actual finding exists. - Req 4: the only imaging on file is a chest X-ray for pneumonia — not lumbar spine imaging. - Req 5: "denies numbness" is a single fragment, not the affirmative structured red-flag screen the policy requires. - No bypass exception applies (symptom duration is 5 days, no progressive deficit or red flag documented) — this case should NOT be authorized on the conservative-therapy-completed path or the bypass-exception path.

PA-2026-72188

Expected: **Unclear** on Req 1, 3, 5 (not confidently Present or Absent). - Req 1: "some conservative measures... exercises and OTC medication" has no dates, provider, or duration — real content, but not specific enough to confirm the 6-week threshold. - Req 3: "mild weakness on the left side" and an unqualified "positive" SLR are real findings but lack the myotome/degree specificity the policy calls for. - Req 5: "nothing concerning noted" is a general reassurance, not the affirmative multi-symptom screen required. - Req 4 (prior imaging) is genuinely absent — no imaging report anywhere in this case.

Total Knee Arthroplasty ([policies/02-total-knee-arthroplasty-policy.md](#))

PA-2026-81004

All requirements should classify as **Present**. - Req 1: X-ray report documents Kellgren-Lawrence Grade 4. - Req 2: NSAID + PT + injection, >3 months, two-plus modalities, all dated. - Req 3: WOMAC 68/100 plus narrative functional limitation. - Req 4: BMI 31.2, below 40, no further documentation needed. - Req 5: infection/contraindication explicitly addressed. - Req 6: separate pre-op clearance note present, patient is 67.

PA-2026-81187

Expected: **Absent** on Req 3 and Req 6. Present on Req 1, 2, 4, 5. - Req 3: no WOMAC/KOOS score and no functional narrative anywhere in this case's documents — the notes go straight from conservative therapy to a surgical recommendation. - Req 6: no pre-operative medical clearance document exists in this case, despite the patient being over 60. - No severe-presentation exception is invoked, so the missing conservative-therapy-completeness exception path doesn't apply here — but note Req 2 (conservative therapy) IS actually satisfied on its own (13 weeks, two modalities), so it should NOT trip.

PA-2026-81299

Expected: **Absent** on Req 1, 2, and 3. - Req 1: the X-ray explicitly shows Kellgren-Lawrence Grade 2 — below the Grade 3-4 threshold — regardless of the patient's own description of the pain as "severe." - Req 2: the only NSAID use is ~2 weeks, and the only physical therapy on file was for the *right* knee two years ago, following a different procedure — not 3 months of *left*-knee conservative care. - Req 3: the WOMAC score of 55 is from 2023 and tied to the right knee, not a current assessment of the left knee under review. - Req 4 and 5 (BMI, infection) are genuinely fine and should be Present.

PA-2026-81364

Expected: **Unclear** on Req 1, 2, and 3. - Req 1: "advanced degenerative joint disease" with no Kellgren-Lawrence grade or specific radiographic feature named. - Req 2: "tried physical therapy and some shots before" — no dates, modality detail, or duration to confirm the 3-month/two-modality threshold. - Req 3: brief narrative ("hurts to walk, hard to do stairs") with no formal functional score. - Req 4 (BMI) is genuinely absent — no current BMI anywhere in this case.

ICD Implantation ([policies/03-icd-implantation-policy.md](#))

PA-2026-91027

All requirements should classify as **Present**. - Req 1: echo dated within 3 months, LVEF 28%, performed after GDMT optimization. - Req 2: GDMT (lisinopril + metoprolol succinate) documented >3 months, reinforced by the pharmacy fill record showing continuous fills with no gaps. - Req 3: NYHA Class II documented. - Req 4: 82 days since the MI, exceeding the 40-day ischemic waiting period, with the specific MI date stated. - Req 5: survival expectation explicitly addressed. - Req 6: no disqualifying recent revascularization or MI-window issue. - Req 7: signed by a board-certified electrophysiologist.

PA-2026-91154

Expected: **Absent** on Req 2 and Req 4. Present on Req 1, 3, 5, 7. - Req 2: GDMT (lisinopril + carvedilol) was started on the SAME date as diagnosis (2026-06-15) — there is no 3-month trial period documented at all; the request is made less than a month later (2026-07-13). - Req 4: for non-ischemic etiology, the required 3-month wait since diagnosis/GDMT initiation has not elapsed — diagnosis was 2026-06-15, request is 2026-07-13 (under 1 month). - Req 1 is actually fine on its own (echo is recent, LVEF qualifies) — this case tests whether the system reads dates relative to each other rather than just checking “is there an LVEF number and is it recent.”

PA-2026-91288

Expected: **Absent** on Req 1, 2, 3, 6, and 7. - Req 1: the qualifying echo is dated 2025-11-20 — about 8 months before the request — far outside the 3-month recency window. No newer echo exists in this case. - Req 2: pharmacy records show both GDMT medications discontinued 2026-02-18, roughly 5 months before the request — not an active/ongoing trial. - Req 3: NYHA Class IV is explicitly excluded by this policy (only Class II/III qualify). - Req 6: a PCI was performed 2 weeks before the request with no repeat LVEF assessment since — this is exactly the exclusion criterion the policy describes. - Req 7: the requesting/signing provider is Family Medicine, not a cardiologist or electrophysiologist. - Req 5 (survival expectation) is genuinely not addressed anywhere either — also Absent.

PA-2026-91356

Expected: **Unclear** on Req 1, 2, 3, and 4. - The note gestures at everything (“EF has been low,” “on heart failure medications for a while,” “seems stable, maybe slightly limited”) without a specific LVEF percentage, drug names/duration, a stated NYHA class, or any waiting-period-relevant dates. Nothing here is specific enough to confirm compliance, but nothing explicitly contradicts it either. - Req 7 (prescriber) IS satisfied — the note is from a cardiologist.

OnabotulinumtoxinA for Chronic Migraine (`policies/04-botox-chronic-migraine-policy.md`)

PA-2026-51012

All requirements should classify as **Present**. - Req 1: diary summary shows ≥ 15 headache days and ≥ 8 migraine-feature days across all 3 months reviewed. - Req 2: two distinct preventive classes (beta-blocker, anticonvulsant), each with dates and discontinuation reasons. - Req 3: ICHD-3 confirmation with explicit red-flag exclusion. - Req 4: age 34 documented. - Req 5: no duplicate therapy, explicitly confirmed. - Req 6: not applicable (initial request), correctly noted as such. - Req 7: board-certified headache medicine neurologist.

PA-2026-51145

Expected: **Absent** on Req 1 and Req 2. - Req 1: “most days of the month” with no diary and no specific day-counts — cannot confirm the $\geq 15/\geq 8$ threshold. - Req 2: only ONE preventive class (topiramate) was trialed, and only for ~6 weeks — short of a second class and short of a meaningful trial duration. - Req 3, 4, 5, 6 are fine on their own terms in this note.

PA-2026-51267

Expected: **Absent** on Req 1, 2, 4, and 5. - Req 1: the diary shows the ≥ 15 -day frequency,

but the patient's own descriptions ("dull, pressing, both sides... no light bothers me") describe tension-type character for most of those days — only 2-3 days/month have migraine features, short of the ≥ 8 threshold. - Req 2: topiramate is the patient's *mother's* medication history, not the patient's own; the propranolol prescription is for an unrelated cardiac indication with no migraine-specific trial — zero qualifying preventive-medication failures for this patient. - Req 4: patient is 16 — a hard exclusion regardless of anything else. - Req 5: cosmetic Botox 3 weeks prior is exactly the duplicate-therapy exclusion. - Req 3 (diagnostic confirmation) is itself questionable given the above, but Req 1/2/4/5 are the clean, unambiguous failures to check for first.

PA-2026-51338

Expected: **Unclear** on Req 1, 2, and 7. - Req 1 & 2: "most days," "a couple of different medications" — real content, but not specific enough (no diary, no drug names, no durations) to confirm compliance. - Req 7: this is a genuinely debatable edge case — the note is authored by a Family Medicine physician *referring to* neurology, not a completed neurology consult and not a headache specialist prescriber. Whether this counts as satisfying, failing, or being Unclear on the prescriber-qualification requirement is worth specifically checking how your system handles it, since the policy language ("PCV with an accompanying neurology consultation note") implies the consult must already exist, not be in progress.